



SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) : 1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☒		Follow-up Case ☐		FOR AMC / NCC USE ONLY																																																																															
A. PATIENT INFORMATION *				Reg. No. / IPD No. / OPD No. / CR No. :																																																																															
1. Patient Initials: Mrs. S		2. Age or date of birth: 56 years old		AMC Report No. :																																																																															
3. Gender: M ☐ F ☒ Other ☐		4. Weight (in Kg.) 68 kg		Worldwide Unique No. :																																																																															
B. SUSPECTED ADVERSE REACTION *				12. Relevant investigations with dates :																																																																															
5. Event / Reaction start date (dd/mm/yyyy)		18/08/2026		13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.) Relevant medical history: Hypertension for 8 years Known allergies: No known drug allergies Past history: No previous episodes of angioedema																																																																															
6. Event / Reaction stop date (dd/mm/yyyy)		19/08/2026																																																																																	
7. Describe Event/Reaction management with details, if any																																																																																			
The patient had been taking enalapril 10 mg once daily for approximately 3 months . Day 0: Enalapril started. Days 1-89: No adverse effects. Day 90: Patient developed sudden lip and tongue swelling approximately 4 hours before arriving at the hospital. Enalapril was suspected as the cause. Enalapril discontinued immediately. Patient was monitored in the emergency department for airway compromise. IV corticosteroid and antihistamine were administered. Oxygen and airway-support equipment were kept ready. Symptoms gradually improved over the next 24 hours. Patient was subsequently discharged. Enalapril was permanently discontinued. An alternative antihypertensive was prescribed.				14. Seriousness of the reaction : No ☐ if Yes ☐ (please tick anyone) ☐ Death (dd/mm/yyyy) ☐ Congenital-anomaly ☐ Life threatening ☐ Disability ☒ Hospitalization-Initial/Prolonged ☐ Other Medically important																																																																															
C. SUSPECTED MEDICATION(S) *				15. Outcome:																																																																															
<table border="1"> <thead> <tr> <th>S. No.</th> <th>8. Name (Brand / Generic)</th> <th>Manufacturer (if known)</th> <th>Batch No. / Lot No.</th> <th>Expiry Date (if known)</th> <th>Dose</th> <th>Route</th> <th>Frequency</th> <th colspan="2">Therapy Dates</th> <th>Indication</th> <th>Causality Assessment</th> </tr> <tr> <th></th> <th></th> <th></th> <th></th> <th></th> <th></th> <th></th> <th></th> <th>Date Started</th> <th>Date Stopped</th> <th></th> <th></th> </tr> </thead> <tbody> <tr> <td>i</td> <td>ENALAPRIL</td> <td></td> <td></td> <td></td> <td>10 mg</td> <td>oral</td> <td>once daily</td> <td>20/05/2026</td> <td>18/08/2026</td> <td>Hypertension</td> <td>Probable/likely</td> </tr> <tr> <td>ii</td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> <tr> <td>iii</td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> <tr> <td>iv[#]</td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> <td></td> </tr> </tbody> </table>				S. No.	8. Name (Brand / Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates		Indication	Causality Assessment									Date Started	Date Stopped			i	ENALAPRIL				10 mg	oral	once daily	20/05/2026	18/08/2026	Hypertension	Probable/likely	ii												iii												iv [#]												☒ Recovered ☐ Recovering ☐ Not Recovered ☐ Fatal ☐ Recovered with sequelae ☐ Unknown							
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9. Action taken after reaction (please tick)				10. Reaction reappeared after reintroduction of suspected medication (please tick)																																																																															
S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown	Yes	No	Effect unknown	Dose (if re-introduced)																																																																									
i	☒								☒																																																																										
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11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)																																																																																			
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i	AMLODIPINE	5 mg	oral	once daily			Hypertension																																																																												
ii	METFORMIN	500 mg	oral	twice daily			Type 2 Diabetes																																																																												
iii [#]	ATORVASTATIN	20 mg	oral	once daily			Dyslipidemia																																																																												
Additional Information :						D. REPORTER DETAILS *																																																																													
Angioedema with swelling of lips and tongue, associated with difficulty swallowing and mild breathing difficulty.						16. Name & Address : <u>XYZ</u> <u>ABC City, 123 State, India</u> Pin : <u>123 456</u> Email : <u>xyz@gmail.com</u> Contact No- : <u>12345 67890</u> Occupation : <u>Pharmacovigilance Associate</u> Signature : _____																																																																													
Signature and Name of Receiving Personnel :						17. Date of this report (dd/mm/yyyy) : <u>25/08/2026</u>																																																																													
Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.																																																																																			

[#] Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

ADVICE ABOUT REPORTING

A. What to report?

All adverse events should be reported

Report non-serious, known or unknown, frequent or rare adverse drug reactions due to Medicines, Vaccines & Herbal Products.

Report every serious adverse drug reactions. A reaction is serious when the patient outcome is :

- Death
- Life-threatening
- Hospitalization (initial or prolonged)
- Disability (significant, persistent or permanent)
- Congenital anomaly
- Report intervention to prevent permanent impairment or damage

NOTE : Serious/Adverse Event following immunization can also be reported in Serious AEFI case Notification Form available on <http://www.ipc.gov.in>

B. Who can report?

All healthcare professionals (Clinicians, Dentists, Pharmacists and Nurse etc.) can report adverse drug reactions

C. Where to report?

Duly filled in Suspected Adverse Drug Reaction Reporting Form can be sent to the nearest Adverse Drug Reaction Monitoring Centre (AMC) or directly to the National Coordination Centre (NCC) for PvPI.

Call on Helpline (Toll Free) 1800 180 3024 to report ADRs or directly mail this filled form to pvpi.ipc@gov.in

A list of nationwide AMCs is available at : <http://www.ipc.gov.in>, http://www.ipc.gov.in/PvPI/pv_home.html

D.What happens to the submitted information?

- Information provided in this form is handled in strict confidence. The causality assessment is carried out at AMCs by using WHO-UMC scale. The analyzed forms are forwarded to the NCC-PvPI through ADR database. Finally the data is analyzed and forwarded to the Global Pharmacovigilance Database managed by WHO Uppsala Monitoring Centre in Sweden.
- The reports are periodically reviewed by the NCC-PvPI. The information generated on the basis of these reports helps in continuous assessment of the benefit-risk ratio of medicines.
- The Signal Review Panel of PvPI reviews the data and suggests any interventions that may be required.

E. Mandatory fields for suspected ADR Reporting Form (*)

Patient initials, age at onset of reaction, reaction term(s), date of onset of reaction, suspected medication(s) & reporter information.

For Adverse Drug Reaction Reporting Tools

- E-mail : pvpi.ipc@gov.in
- PvPI Helpline (Toll Free) : 1800 180 3024 (9:00 AM to 5:30 PM, Monday-Friday)
- ADR Mobile App : "ADRPvPI"